

Risk Summary

Preeclampsia

Medium Score: 0.6395082154771051

Model Interpretation: Risk-based triage only (not diagnosis).

Macrosomia

Medium Score: 0.5943816940585895

Model Interpretation: Risk-based coaching support.

Summary: The patient is at medium risk for preeclampsia and macrosomia. Key findings include elevated blood pressure (142/79 mmHg) and positive proteinuria (1), warranting an alert to the doctor for further evaluation of preeclampsia risk. For macrosomia risk, pre-pregnancy BMI is high, and fasting glucose is at the upper end of the normal range, suggesting a need for continued focus on lifestyle management.

Patient Snapshot (Recent Inputs)

Parameter	Value	Notes
Blood Pressure	142/79 mmHg	Hypertensive threshold ≥140/90, severe ≥160/110.
Proteinuria (dipstick)	1	≥1+ supports PE suspicion in context of HTN.
Symptoms	Headache=0, Vision=0, RUQ Pain=0, Swelling=0	Severe features increase escalation priority.
Glucose	Fasting=99 mg/dL 1hr PP=126 mg/dL	Targets typically fasting <95, 1hr <140 (pregnancy goals).
HbA1c	5.6	Higher HbA1c indicates sustained glycemic elevation.
Maternal Context	Age=38 BMI=42.4 GDM=0	Risk modifiers for fetal overgrowth + metabolic risk.
Activity / Sleep	Post-meal walk=29 min Sleep=6.1 hrs	Lifestyle factors influence glycemic response.
Food Log	paneer salad	Used for dietary correlation patterns.

Reason for Alert

Action Requested: ALERT_DOCTOR

Clinical Rationale: ALERT: Patient TEST_MED_MED (32 weeks gestational age) has a MEDIUM preeclampsia risk tier (score 0.64) and MEDIUM macrosomia risk tier (score 0.59). Key vitals for preeclampsia include blood pressure of 142/79 mmHg and proteinuria_dipstick of 1. No severe symptoms (headache, vision changes, abdominal pain, facial/hand swelling) reported. For macrosomia, pre-pregnancy BMI is 42.4, fasting glucose is 99 mg/dL, and GDM is not diagnosed. Review is recommended for elevated BP and proteinuria suggestive of preeclampsia risk.

Suggested Next Steps

- Review trend: BP, symptoms, proteinuria, glucose readings
- Consider confirmatory tests (protein/creatinine ratio, LFTs, platelets) if clinically indicated
- Consider GDM diet review or insulin adjustment if glucose targets exceeded
- Provide patient education + return precautions

⚠ This report is generated by an AI decision-support system and does not replace clinical judgment. Always follow institutional protocols.